

Document Reference Number UHN-PO-HR10

Policy/Guideline Title:	Leave in Relation to Starting a Family Policy		
Executive Summary:	University Hospitals of Northamptonshire NHS Group is committed to ensuring that all colleagues who are applying for leave in relation to maternity, paternity, adoption or shared parental leave have access to the leave they are entitled to through statutory regulations and national NHS terms and conditions of service. In addition to these entitlements this policy details, or signposts to the processes to be followed and the support and additional leave that is available should colleagues require it in relation to baby loss, for babies that are receiving neonatal care within the first 28 days or for IVF treatment.		
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Ratified by:			
Received for Information:			

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Dedicated to Excellence Values	What our values mean	How this applies to the policy
 Compassion	We care about our patients and each other. We consistently show kindness and empathy and take the time to imagine ourselves in other people's shoes.	- This policy should be applied with compassion, genuinely caring for the well-being of our colleagues. - Managers are to consistently show kindness and empathy, taking the time to understand colleagues challenges, needs and personal circumstances
 Accountability	We take responsibility for our decisions, our actions and our behaviours. We do what we say we will do, when we say we will do it. We acknowledge our mistakes and we learn.	Colleagues and managers take responsibility for their decisions, actions, and behaviours, ensuring that they fulfil their commitments and meet expectations in relation to their duties and the application of this policy.
 Respect	We value each other, embrace diversity and make sure everyone feels included. We take the time to listen to, appreciate and understand the thoughts, beliefs and feelings of others.	Colleagues and managers are asked to respect the measures put into place to ensure the application of this policy is applied fairly and equitably with the aim of safeguarding the overall health and safety, welfare of our workforce and fostering a healthy and respectful workplace for all our colleagues and our patients.
 Integrity	We are consistently open, honest and trustworthy. We can be relied upon, we stand by our values and we always strive to do the right thing.	We consistently demonstrate openness, honesty, and trustworthiness when implementing this policy and its processes.
 Courage	We dare to take on difficult challenges and try out new things. We find the strength to speak up when it matters and we see potential failure as an opportunity to learn and improve.	We find the strength to speak up when it matters, constructively sharing our opinions and concerns, if we feel that this policy has not been applied in a fair and consistent manner.

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POLICY

1. INTRODUCTION

This policy provides guidance for you and your line manager/supervisor regarding the rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and any additional provisions available from the University Hospitals of Northamptonshire NHS Group (UHN) in relation to:

Type of Leave / Provision	Section
Maternity	4.1
Paternity	4.2
Adoption	4.3
Shared Parental Leave	4.4
Surrogacy	4.5
Baby Loss	4.6
IVF / Fertility Treatment	4.7
Risk Assessments	Appendix 1 & 2

You can also find additional information [here](#) in the Leave to Start a Family SWAY.

Further information can also be found on the intranet

- Northampton General Hospital (NGH) - [Maternity, Adoption, Paternity and Shared Parental Leave](#)
- Kettering General Hospital (KGH) – [Maternity, Paternity and Adoption Leave](#)

Guidance can be obtained from the following colleagues in the People Directorate:

NGH Colleagues	KGH Colleagues
People Services – Governance & Compliance Lead ngh-tr.hradvisors@nhs.net	People HR Advisors kgg-tr.humanresourcesupport@nhs.net

Other relevant policies you should make yourself familiar with include:

- Agile, Flexible and Hybrid Working Policy – should you wish to consider applying for a change to your hours or working patterns following your leave.
- Annual Leave & Public Holiday Entitlements for Agenda for Change Staff Policy – as you will continue to accrue your annual leave and bank holidays as normal during your leave. More details are provided in this policy regarding this.
- Appraisal Policy – should you be due an appraisal whilst you are on leave.
- Health and Wellbeing at Work Policy (including Supporting Attendance at Work) – should you be unfit for work during your pregnancy.

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- Special Leave Policy – should you wish to access alternative types of leave at a later date, such as parental leave or emergency leave.

2. PURPOSE

The purpose of this policy is to inform you of your rights and responsibilities, and the processes and timescales you need to follow to apply for these types of leave.

It also provides information about your line managers/supervisors and our UHN responsibilities to you, along with the processes we need to follow to comply with the statutory legislation and National Terms and Conditions of Service.

3. SCOPE

This policy covers all colleagues, regardless of gender, who are employed under a contract of employment with either NGH, KGH or UHN. It also applies to Bank Worker colleagues, who may be entitled to some statutory provisions.

4. SUBSTANTIVE CONTENT

4.1. Maternity Leave

4.1.1 Maternity Rights

Under statutory regulations and NHS Terms and Conditions of Service your rights as a pregnant colleague are:

- Paid time off for antenatal care
- Maternity leave
- Maternity pay if you meet the qualifying criteria or Maternity Allowance
- Protection against unfair treatment, discrimination or dismissal, for reasons related to your pregnancy.

Once your maternity leave has commenced you also have the right to:

- Keeping in touch days
- Protection of your terms and conditions of service
- The right to return to your job on no less favourable terms
- Redundancy protection up to 18 months from the date your baby is born or expected week of childbirth
- The same redundancy rights as your colleagues and you must be offered suitable alternative employment, if available
- Access shared parental leave entitlements if you meet the qualifying criteria.

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In addition, you and new applicants (internal or external) for posts within your Trust/UHN must be given equal opportunities in employment, promotion, training and development, regardless of if you are pregnant or on maternity leave.

Your maternity leave counts as continuous service and you retain all of your contractual rights, except your normal salary whilst you are on maternity leave.

4.1.2 Risk Assessments

All organisations have a statutory obligation, in line with the Management of Health and Safety at Work Regulations 1999, to protect the health and safety of new and expectant employees including those who are breast/chestfeeding.

Therefore, once you have informed your line manager/supervisor of your pregnancy, they are required to undertake an individual risk assessment, with you, as soon as possible to identify if there are any hazards and assess the risks that may affect the safety of you and/or your baby.

Action must be taken to remove, reduce or control any risks that have been identified.

In conjunction with you, your risk assessment should be undertaken with somebody that understands your role, your working environment, and the tasks/duties that you undertake.

The risk assessment should be repeated, regularly and we recommend it is reviewed at the beginning of each trimester:

- 2nd Trimester – week 13
- 3rd Trimester – week 28

It should also be repeated if you have any concerns at all, at any point during your pregnancy, about risks or hazards at work that could potentially put yourself or your baby at risk.

Risk assessments for Bank Workers should be carried out by a supervisor in the area that you are undertaking your bank shifts. That means if you bank in more than one area, you will require a risk assessment for each area. If necessary your Bank Co-ordinator will liaise with the area/s you undertake your bank work to assign someone to undertake your risk assessments.

Risk assessments for Medical and Dental colleagues should be carried out by a Clinical Supervisor. If you do not have a Clinical Supervisor your area's Clinical Director should assign someone to undertake your risk assessment/s and to be a point of contact for you, for support during your pregnancy.

Full details of the risk assessment process and associated forms are at Appendix 1 & 2.

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4.1.3 Paid Time Off for Antenatal Care

Once you have told us you are pregnant you will be entitled to reasonable paid time off work for antenatal care, without the need to make the time up or take it as annual leave. We cannot refuse to give reasonable time off, however your line manager/supervisor is entitled to ask for evidence of the appointments from the second appointment onwards.

We ask that you give as much notice as you possibly can to ensure that your department can make adequate arrangements to cover your duties during your appointment.

Antenatal care is not just medical appointments it can include relaxation or parent craft classes, medical examinations/scans, if these are recommended by a doctor, midwife, or medical practitioner.

4.1.4 Maternity Leave

As an NHS employee you have the right to take 52 weeks of maternity leave, regardless of length of service or if you are in a contracted post or a Bank Worker.

You must take the compulsory maternity leave period of two weeks immediately after your baby is born.

The earliest you can plan to start your maternity leave is from the start of week 29, unless your baby is born early. It can start on any day of the week. The latest date you can plan to start your maternity leave is your due date.

If your baby is born before the date it has been agreed your maternity leave is to start, the maternity leave start date will need to be adjusted to the date after the baby is born and you should notify your line manager/supervisor as soon as possible in order for this to be organised.

Your maternity leave will start automatically if you are off work for a pregnancy related illness in the 4 weeks before the baby is due, regardless of what has been previously agreed. Section 4.1.6 gives more detail about this.

You must take your maternity leave all in one go, you cannot break or split your maternity leave.

If after submitting your paperwork you want to change the start date of your maternity leave you need to give 4 weeks' notice, or as much as you can, if that is not possible.

If you sadly lose your baby please see section 4.6 for more information on the leave options that are available.

You can choose to end your maternity leave early to access shared parental leave (see section 4.4).

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4.1.5 Maternity Pay

You may be entitled to maternity pay from your employing Trust, if you meet the qualifying criteria which is based on specific eligibility criteria as detailed in the table below:

Pay Type	Qualifying Criteria	Pay Received
Occupational Maternity Pay (OMP)	- Have at least 12 months continuous service (in a substantive post – excludes bank work) with one or more NHS employers at the beginning of the 11th week before the expected week of childbirth. - Have notified us you are pregnant before the end of the 15th week before the expected date of childbirth, or as or as soon as is reasonably practical if this is not possible. - Intend to return to work in the NHS (in a substantive post – excludes bank work), for at least 3 months after your maternity leave has ended.	8 weeks at full pay (less any SMP / Maternity Allowance). 18 weeks at half pay. 26 weeks unpaid maternity leave
Statutory Maternity Pay (SMP)	- Have worked for your employing Trust continuously for at least 26 weeks at the 15th week before the expected week of childbirth. This period must include at least 1 days employment in the qualifying week. - Provide the correct notice and proof of pregnancy - Have average weekly earnings in the 8 weeks up to and including the qualifying week of at least equal to the Lower Earnings Limit for National Insurance contributions.	6 weeks at 90% of your average weekly earnings*. 33 weeks SMP or 90% of your average weekly earnings, whichever is lower*. 13 weeks unpaid maternity leave *Please check Gov.uk website for current Statutory Rates
Combined Occupational & Statutory Maternity Pay	Have qualified for both occupational and statutory maternity pay.	8 weeks full pay (inclusive of SMP). 18 weeks half pay plus SMP 13 weeks SMP 13 weeks unpaid maternity leave

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Pay Type	Qualifying Criteria	Pay Received
Maternity Allowance (MA)	Have not qualified for statutory maternity pay because you have less than 26 weeks continuous service with your employing Trust at the 15th week prior to the expected week of childbirth or do not have earnings equal to the lower earnings limit for National Insurance contributions.	- No statutory maternity pay - Can apply for Maternity Allowance via Maternity Allowance: How to claim - GOV.UK (www.gov.uk)

If you are a Bank Worker who works regularly you may be entitled to statutory maternity pay, if you meet all the qualifying conditions. You will not be entitled to occupational maternity pay.

Your maternity pay will be calculated on the average weekly earnings (contractual earnings which count for NI contributions) that occur over the 'relevant period' of at least eight weeks up to and including the last pay day before the end of the qualifying week (15th week before the expected week of childbirth). This period will vary depending on the frequency of when you are paid and can also be affected by additions/deductions to your contractual earnings which count for NI contributions salary during the 'relevant period'. This means if you are in a salary sacrifice scheme or have deductions for childcare vouchers your average pay calculation will be affected.

If you are entitled to occupational maternity pay, it may be possible for this to be paid as an equal fixed amount across your maternity leave period. However please note that choosing this option:

- Only applies to occupational maternity pay only (8 weeks full pay and 18 weeks half pay). Therefore, please note that equaling out OMP is likely to result in your salary being different each month.
- It is not possible to equal out pension contribution deductions, so this again may result in your salary being different each month.

If you are absent due to sickness during the 'relevant period' for the average pay calculation and as a result are in a no pay or half pay situation, your average weekly earnings calculation will be based on notional full sick pay.

If you are taking two periods of maternity leave close together and during the 'relevant period' you are in receipt of maternity pay (such as half pay/SMP) or in the no pay period. Your average weekly earnings calculation will be based on notional full pay.

If you receive a pay award or increment, after the average weekly pay calculation has taken place your maternity pay will be recalculated. You should refer to the Staff Appraisal Policy for full details about increments that are due to take place during the maternity leave period. However, you should move to the increment by default, and an appraisal should take place as soon as possible after your return, if one could not be undertaken before you started your maternity leave.

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Your maternity pay is subject to all statutory deductions (tax and national insurance) and pension, if applicable. During any unpaid maternity leave your contributions will be accrued until you return to work. This means that upon return to work, your accrued contributions are then re-paid over the same time period as your unpaid leave.

Any other deductions from your salary, such as parking, the staff lottery or union subscriptions will also continue to be made, unless you make alternative arrangements directly with the appropriate departments/organisation. No monies will be deducted whilst you are on unpaid leave, so you must contact the relevant body to make alternative arrangements, regarding any arrears to be paid on your return to work.

If you have any relevant agreements to repay any loans or monies owed to your Trust/UHN, you are obliged to continue these arrangements during your maternity leave, unless you agree alternative arrangements.

Your maternity pay, excluding Maternity Allowance will be paid directly into your nominated bank account on normal Trust pay day. There is no requirement for you to complete your timesheet via e-Rostering whilst you are on maternity leave.

4.1.6 Sickness / Illness

If you are off work with a non-pregnancy related illness you will normally be able to take sick absence, in line with the Health and Wellbeing at Work Policy (including Supporting Attendance at Work Policy), until the date it has been agreed you are starting your maternity leave.

If, however, your illness is pregnancy-related and you are within the 4 week period before your due date (i.e. the start of week 36 onwards), your maternity leave period will start immediately from your first day of sickness, regardless of what has already been agreed. Odd days of pregnancy-related illness during this period may be disregarded if you wish to continue working until the maternity leave start date you have previously agreed.

If you are on long-term pregnancy related sickness absence and it is known that you will not be returning to work before the start of week 36 of your pregnancy you will need to commence your maternity leave no later than the start of week 36 (Sunday).

Normal sick pay arrangements apply, as per the National NHS Terms and Conditions of Service and the Health and Wellbeing at Work Policy (including Supporting Attendance at Work), during any periods of sickness whilst pregnant.

If you are unable to attend work due to sickness when your maternity leave has ended, the normal contractual arrangements for sickness absence will apply.

4.1.7 Fixed Term Contracts

If you have a fixed term or rotational training contract which expires after the 11th week before the expected week of childbirth you will have your contract of employment extended to allow you to receive the 52 weeks maternity leave plus any pay you are entitled to,

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subject to meeting the other qualifying conditions. You will not be required to repay any occupational maternity pay if you are not returning to your Trust following your maternity leave due your contract having ended, as it would have done, should your pregnancy have not occurred.

4.1.8 Rotational Training Contracts

Where you are on a planned rotational training contract with appointments with one or more NHS employers as part of an agreed programme of training, you shall have the right to return to work in the same post or in the next planned post irrespective of whether your contract would otherwise have ended if pregnancy and childbirth had not occurred. In these circumstances your contract will be extended to enable you to complete the agreed programme of training.

In addition if you have changed your employing Trust as a requirement of your training programme and therefore do not have sufficient statutory service to be eligible for statutory maternity pay, because of your rotational training programme, you will receive the value of statutory maternity pay from your Trust.

4.1.9 Babies Requiring Neonatal Care Within 28 Days After Birth

For babies born on or after 6 April 2025 The Neonatal Care (Leave and Pay) comes into force and provides additional leave and pay for parents, with responsibility for raising the baby, if the baby has received any medical or palliative neonatal care for at least 7 consecutive days within the first 28 days after their birth, which is defined as:

1. Any medical care received in hospital.
2. Medical care received elsewhere following discharge from an inpatient stay hospital. The care must be under the direction of a consultant and includes ongoing monitoring and visits to the child by healthcare professionals.
3. Palliative or end of life care.

You can take one weeks Neonatal Care Leave for each uninterrupted week (7 day period) of care that your baby receives, up to a maximum of 12 weeks, following at least 7 full days of care. For example:

Number of Weeks (7 days) Baby Receives Neonatal care	Total Number of weeks of Neonatal Care Leave Available
6 days	0 weeks
1 week	1 week
5 weeks	5 weeks
9 weeks	9 weeks
12 weeks	12 weeks
15 weeks	12 weeks

As you will already be on Maternity Leave you will be able to apply to take Tier 2 period neonatal care leave immediately after your maternity leave finishes. It must be taken within

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68 weeks of the baby's birth, taken in complete weeks and as one continuous block of leave. It also needs to be used to care for your baby.

To apply for this leave you will need to complete the Neonatal Care Leave Application Form (Tier 2) which can be found via:

- NGH – [Maternity, Adoption, Paternity and Shared Parental Leave](#)
- KGH – [Maternity, Paternity and Adoption Leave](#)

If you are taking one weeks leave you must give at least 2 weeks' notice before the first day of the neonatal care leave. If you are taking two or more weeks leave you must give at least 4 weeks' notice. However, in order not to miss any payroll deadlines we would always recommend at least 4 weeks' notice for this type of leave.

Under the Neonatal Act the provision for pay is at the same rate as Statutory Maternity Pay. However, we have decided to retain our previous provision of paying this leave at full pay and it will be recorded as Neonatal Care Leave (paid), within the Parenting section of e-Roster.

4.1.10 Process for Applying for Maternity Leave / Key Dates

The flowchart below summarises the key responsibilities of you and your line manager/supervisor/employing Trust and the key pregnancy dates as part of the maternity process.

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4.1.11 More Information

You will find additional information [here](#) in the Leave to Start a Family SWAY, which covers:

- Annual leave
- Keeping in Touch Days
- Returning to Work
- Breast/Chestfeeding

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- Tax-free Childcare
- Day Nurseries

You can also access UHN's 'Maternity Handbook – Easy Reference Guide to Maternity Leave for Colleagues and Managers' via the intranet:

- NGH – [Maternity, Adoption, Paternity and Shared Parental Leave](#)
- KGH – [Maternity, Paternity and Adoption Leave](#)

In the Maternity Handbook you will find lots of information about maternity leave include a guide to key dates and a checklist for pregnant colleagues and a checklist for managers/supervisors.

Further assistance can be sought from:

NGH Colleagues	KGH Colleagues
People Services – Governance & Compliance Lead ng-h-tr.hradvisors@nhs.net	People HR Advisors kgh-tr.humanresourcesupport@nhs.net

4.2. Paternity Leave and Pay

4.2.1 Paternity Rights

Under statutory regulations and NHS Terms and Conditions of service if you are either the:

- The biological parent of the child, regardless of gender, who has parental responsibility
- Spouse or partner of the person with parental responsibility (including same sex partners)
- Child's adopter
- Spouse or partner (including same-sex relationships) of the child's adopter
- An intended parent if you are having a child through a surrogacy arrangement.

You have the right to:

- Paid time off to attend antenatal care or essential adoption appointments
- Paternity leave
- Paternity pay, if you meet the qualifying criteria
- Protection against unfair treatment, discrimination or dismissal, for reasons related to your paternity leave
- Protection of your terms and conditions of service
- The right to return to your job on no less favourable terms
- The same redundancy rights as your colleagues and you must be offered suitable alternative employment, if available
- Access shared parental leave entitlements, if you meet the qualifying criteria.

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In addition, you and new applicants (internal or external) for posts within your Trust/UHN must be given equal opportunities in employment, promotion, training and development, regardless of if you are on paternity leave.

Your paternity leave counts as continuous service and you retain all of your contractual rights, except your normal salary whilst you are on paternity leave.

4.2.2 Paid Time Off for Antenatal Care/Adoption Appointments

Once you have told us that you will be applying for paternity leave you will be entitled to reasonable paid time off work for antenatal care or essential adoption meetings when you are being assessed for adoption, without the need to make the time up or take it as annual leave. We cannot refuse to give reasonable time off, however your line manager/supervisor is entitled to ask for evidence of the appointments from the second appointment onwards.

We ask that you give as much notice as you possibly can to ensure that your department can make adequate arrangements to cover your duties during your appointment.

4.2.3 Paternity Leave

As an NHS employee you have the right to take 1 or 2 weeks paternity leave, regardless of length of service or if you are in a contracted post or a Bank Worker:

You can take a single period of leave for either one or two weeks, or you can split the leave and take two periods of one week at different times. This can be taken up to 52 weeks after the birth of your child or their placement date, in the case of adoption.

The leave must be taken in complete weeks, it cannot be taken as odd days off.

You cannot start your paternity leave before your baby is born, or a child is placed with you in the case of adoption. Therefore, the earliest you can plan to start your paternity leave is the due date or placement date.

If your baby is born or the child's placement starts before the date it has been agreed your paternity leave is to start, the paternity leave start date will need to be adjusted if you wish to start your paternity leave earlier, depending on the circumstances. You should notify your line manager/supervisor as soon as possible in order for this to be organised.

If your baby's birth or child's placement is delayed, you will need to change the start date of your paternity leave, as it cannot commence before the birth or placement. You should notify your line manager/supervisor as soon as possible in order for this to be organised.

If after submitting your paperwork you want to change the start date of your paternity leave you need to give 4 weeks' notice, or as much as you can, if that is not possible.

If you sadly lose your baby please see section 4.6 for more information on the leave options that are available.

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In the case of adoption, should the adoption break down (“be disrupted”) you will be entitled to continue your paternity leave.

If you are also taking shared parental leave (see section 4.4) you must take your paternity leave first.

4.2.4 Paternity Pay

You may be entitled to paternity pay from your employing Trust, if you meet the qualifying criteria which is based on specific eligibility criteria as detailed in the table below:

Pay Type	Qualifying Criteria	Pay Received
Occupational Paternity Pay (OPP)	- Have at least 12 months continuous service (in a substantive post – excludes bank work) with one or more NHS employers at the beginning of the expected week of childbirth or the week that you were notified of being matched with a child. - Have notified us of the pregnancy, adoption or surrogacy before the end of the 15th week before the expected date of childbirth/placement, or as or as soon as is reasonably practical if this is not possible.	1 or 2 weeks at full pay (less any SPP or other allowances)
Statutory Paternity Pay (SPP)	- Have worked for your employing Trust continuously for at least 26 weeks at the 15th week before the expected week of childbirth or the week that you were notified of being matched with a child. This period must include at least 1 days employment in the qualifying week. - Provide the correct notice and proof of the pregnancy, adoption or surrogacy - Have average weekly earnings in the 8 weeks up to and including the qualifying week of at least equal to the Lower Earnings Limit for National Insurance contributions. - Be taking time off to look after your child and responsible for the child's upbringing	1 or 2 weeks SPP or 90% of your average weekly earnings, whichever is lower* *Please check Gov.uk website for current Statutory Rates
Combined Occupational & Statutory Paternity Pay	Have qualified for both occupational and statutory paternity pay.	1 or 2 weeks full pay (inclusive of SPP).

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If you are a Bank Worker who works regularly you may be entitled to statutory paternity pay, if you meet all the qualifying conditions. You will not be entitled to occupational paternity pay.

Your paternity pay is subject to all statutory deductions (tax and national insurance) and pension, if applicable.

Any other deductions from your salary, such as parking, the staff lottery or union subscriptions will also continue to be made, unless you make alternative arrangements directly with the appropriate departments/organisation.

If you have any relevant agreements to repay any loans or monies owed to your Trust/UHN, you are obliged to continue these arrangements during your paternity leave, unless you agree alternative arrangements.

Your paternity pay will be paid directly into your nominated bank account on normal Trust pay day. There is no requirement for you to complete your timesheet via e-Rostering whilst you are on paternity leave.

4.2.5 Fixed Term Contracts

If you have a fixed term or rotational training contract which expires before the birth, you will not qualify for paternity leave. If your contract ends after the birth you will qualify for paternity leave plus any pay you are entitled to, subject to meeting the other qualifying conditions. You will not be required to repay any occupational paternity pay if you are not returning to your Trust following your paternity leave due your contract having ended, as it would have done, should your paternity leave have not occurred.

4.2.6 Rotational Training Contracts

Where you are on a planned rotational training contract with appointments with one or more NHS employers as part of an agreed programme of training, you shall have the right to return to work in the same post or in the next planned post irrespective of whether your contract would otherwise have ended if paternity leave had not occurred. In these circumstances your contract will be extended to enable you to complete the agreed programme of training, if required.

In addition if you have changed your employing Trust as a requirement of your training programme and therefore do not have sufficient statutory service to be eligible for statutory paternity pay, because of your rotational training programme, you will receive the value of statutory paternity pay from your Trust.

4.2.7 Babies Requiring Neonatal Care Within 28 Days After Birth

For babies born on or after 6 April 2025 The Neonatal Care (Leave and Pay) comes into force and provides additional leave and pay for parents, with responsibility for raising the baby, if the baby has received any medical or palliative neonatal care for at least 7 consecutive days within the first 28 days after their birth, which is defined as:

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4. Any medical care received in hospital.
5. Medical care received elsewhere following discharge from an inpatient stay hospital. The care must be under the direction of a consultant and includes ongoing monitoring and visits to the child by healthcare professionals.
6. Palliative or end of life care.

You can take one weeks Neonatal Care Leave for each uninterrupted week (7 day period) of care that your baby receives, up to a maximum of 12 weeks, following at least 7 full days of care. For example:

Number of Weeks (7 days) Baby Receives Neonatal care	Total Number of weeks of Neonatal Care Leave Available
6 days	0 weeks
1 week	1 week
5 weeks	5 weeks
9 weeks	9 weeks
12 weeks	12 weeks
15 weeks	12 weeks

Following your paternity leave, if your baby is still receiving care, you will be able to request Tier 1 period neonatal care leave. It can be taken in one week blocks and does not have to be taken continuously. It does need to be used to care for your baby and can extend to the 7th day after your baby stops receiving neonatal care.

To request this leave you need to give your line manager as much notice as possible before your first day of absence. Notice for Tier 1 leave does not need to be in writing and it will be recorded as Neonatal Care Leave (paid), within the Parenting section of e-Roster.

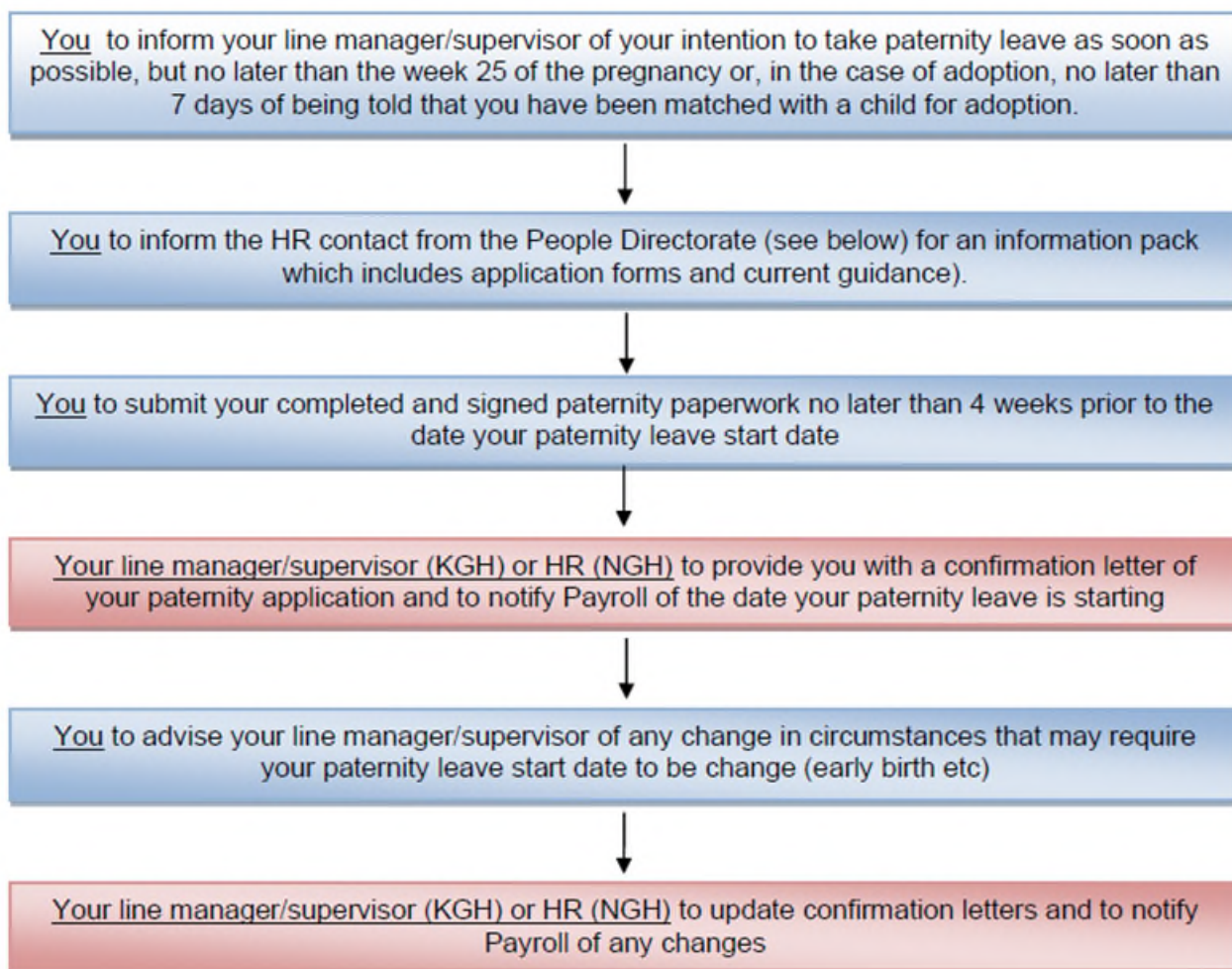
If following the end of your baby's neonatal care you have not exhausted the number of weeks available to you, you will be able to apply to take the remaining weeks as Tier 2 period neonatal care leave. It must be taken within 68 weeks of the baby's birth, taken in complete weeks and as one continuous block of leave. It also needs to be used to care for your baby.

Under the Neonatal Act the pay provision for neonatal care leave is at the same rate as Statutory Paternity Pay. However, we have decided to retain our previous provision of paying this leave at full pay and it will be recorded Neonatal Care Leave (paid), within the Parenting section of e-Roster.

4.2.8 Process for Applying for Paternity Leave / Key Dates

The flowchart below summarises the key responsibilities of you and your line manager/supervisor/employing Trust and the key dates as part of the paternity process.

POLICY



4.2.9. More Information

You will find additional information here in the Leave to Start a Family SWAY, which covers:

- Annual leave
- Tax-free Childcare

Day Nurseries

You can also access further information via the intranet:

- NGH – [Maternity, Adoption, Paternity and Shared Parental Leave](#)
- KGH – [Maternity, Paternity and Adoption Leave](#)

Further assistance can be sought from:

NGH Colleagues	KGH Colleagues
People Services – Governance & Compliance Lead ng-h-tr.hradvisors@nhs.net	People HR Advisors kg-h-tr.humanresourcesupport@nhs.net

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4.3. Adoption Leave and Pay

4.3.1 Adoption Rights

Under statutory regulations and NHS Terms and Conditions of service if you are either:

- Adopting a child through a UK Adoption Agency, or
- Fostering to adopt through a UK Adoption Agency, or
- The intended parent through a surrogacy arrangement

You have the right to:

- Paid time off to attend essential adoption meetings
- Adoption leave
- Adoption pay, if you meet the qualifying criteria
- Protection against unfair treatment, discrimination or dismissal, for reasons related to your adoption leave

Once your adoption leave has commenced you also have the right to:

- Keeping in touch days
- Protection of your terms and conditions of service
- The right to return to your job on no less favourable terms
- Redundancy protection period from the date your adoption leave begins and will end 18 months either from the date your adoption placement starts or the date your child enters England, Scotland or Wales, if it is an overseas adoption
- The same redundancy rights as your colleagues and you must be offered suitable alternative employment, if available
- Access shared parental leave entitlements, if you meet the qualifying criteria.
- Access to leave under the Neonatal Care (Leave and Pay) Act should an adopted baby receive neonatal care within the first 28 days of birth for at least 7 days (see section 4.1.9 – Maternity or section 4.2.7 – Paternity)

If you are adopting as a couple only one person can take adoption leave. The other partner can take paternity leave, if they meet the qualifying criteria (see section 4.2).

Please note that different rules apply if you are adopting a child from overseas, in these circumstances please contact HR/People Team for more information via the contact details in section 1.

You should also note that the provisions under this policy do not apply if you are becoming a special guardian or kinship carer, adopting a family member (such as a stepchild) or adopting privately, e.g. without permission from a UK authority or adoption agency. However, you should speak to your line manager to see if other arrangements can be facilitated to support you during this time such as working from home, taking time owing, flexitime, using further annual leave or unpaid leave or making the hours back (by a date agreed between you and your line manager/supervisor). You could also consider applying

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for an employment/career break and more information on this can be found in the Special Leave Policy.

In addition, you and new applicants (internal or external) for posts within your Trust/UHN must be given equal opportunities in employment, promotion, training and development, regardless of if you on adoption leave.

Your adoption leave counts as continuous service and you retain all of your contractual rights, except your normal salary whilst you are on adoption leave.

4.3.2 Paid Time Off for Adoption Appointments

You are entitled to reasonable paid time off work for essential adoption meetings when you are being assessed for adoption, without the need to make the time up or take it as annual leave. We cannot refuse to give reasonable time off, however your line manager/supervisor is entitled to ask for evidence of the appointments from the second appointment onwards.

We ask that you give as much notice as you possibly can to ensure that your department can make adequate arrangements to cover your duties during your appointment.

4.3.3 Adoption Leave

As an NHS employee you have the right to take 52 weeks of adoption leave, regardless of length of service or if you are in a contracted post or a Bank Worker.

If you are adopting you can start adoption leave anytime (and on any day of the week) from 14 days before the date of the placement. You must have started your adoption leave no later than the placement date.

If you are using a surrogate to have a child you can start your adoption leave the day, or the day after the child is born.

You must take your adoption leave all in one go, you cannot break or split your adoption leave.

If after submitting your paperwork you want to change the start date of your adoption leave you need to give 4 weeks' notice, or as much as you can, if that is not possible.

Should the adoption break down ("be disrupted") you will be entitled to continue your adoption leave.

You can choose to end your adoption leave early to access shared parental leave (see section 4.4).

4.3.4 Adoption Pay

You may be entitled to adoption pay from your employing Trust, if you meet the qualifying criteria which is based on specific eligibility criteria as detailed in the table below:

POLICY

Pay Type	Qualifying Criteria	Pay Received
Occupational Adoption Pay (OAP)	- Have at least 12 months continuous service (in a substantive post – excludes bank work) with one or more NHS employers at the beginning of the week in which you are notified of being matched with a child for adoption, or by the 15th week before the baby's due date if applying via a surrogacy arrangement - Intend to return to work in the NHS (in a substantive post – excludes bank work), for at least 3 months after your adoption leave has ended.	8 weeks at full pay (less any SAP). 18 weeks at half pay. 26 weeks unpaid adoption leave
Statutory Adoption Pay (SAP)	- Have worked for your employing Trust continuously for at least 26 weeks for any day in the week that you were matched with a child or by the 15th week before the baby's due date if applying via a surrogacy arrangement. This period must include at least 1 days employment in the qualifying week. - Provide the correct notice and proof of the adoption or surrogacy - Have average weekly earnings in the 8 weeks up to and including the qualifying week of at least equal to the Lower Earnings Limit for National Insurance Contributions.	6 weeks at 90% of your average weekly earnings* 33 weeks SAP or 90% of your average weekly earnings, whichever is lower* 13 weeks unpaid adoption leave *Please check Gov.uk website for current Statutory Rates
Combined Occupational & Statutory Adoption Pay	Have qualified for both occupational and statutory adoption pay.	8 weeks full pay (inclusive of SAP). 18 weeks half pay plus SAP 13 weeks SAP 13 weeks unpaid adoption leave

If you are a Bank Worker who works regularly you may be entitled to statutory adoption pay, if you meet all the qualifying conditions. You will not be entitled to occupational adoption pay.

Your adoption pay will be calculated on the average weekly earnings (contractual earnings which count for NI contributions) that occurs over a 'relevant period' of at least eight weeks

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up to and including the last pay day before the end of the week that matching took place. This period will vary depending on the frequency of when you are paid and can also be affected by additions/deductions to your contractual earnings which count for NI contributions salary during the 'relevant period'. This means if you are in a salary sacrifice scheme or have deductions for childcare vouchers your average pay calculation will be affected.

If you are entitled to occupational adoption pay, it may be possible for this to be paid as an equal fixed amount across your adoption leave period. However please note that choosing this option:

- a) Only applies to occupational adoption pay only (8 weeks full pay and 18 weeks half pay). Therefore, please note that equalling out OAP is likely to result in your salary being different each month.
- b) It is not possible to equal out pension contribution deductions, so this again may result in your salary being different each month.

If you are absent due to sickness during the 'relevant period' for the average pay calculation and as a result are in a no pay or half pay situation, your average weekly earnings calculation will be based on notional full sick pay.

If you are taking two periods of adoption leave close together and during the 'relevant period' you are in receipt of adoption pay (such as half pay/SAP) or in the no pay period. Your average weekly earnings calculation will be based on notional full pay.

If you receive a pay award or increment, after the average weekly pay calculation has taken place your adoption pay will be recalculated. You should refer to the Staff Appraisal Policy for full details about increments that are due to take place during the adoption leave period. However, you should move to the increment by default, and an appraisal should take place as soon as possible after your return, if one could not be undertaken before you started your adoption leave.

Your adoption pay is subject to all statutory deductions (tax and national insurance) and pension, if applicable. During any unpaid adoption leave your contributions will be accrued until you return to work. This means that upon return to work, your accrued contributions are then re-paid over the same time period as your unpaid leave.

Any other deductions from your salary, such as parking, the staff lottery or union subscriptions will also continue to be made, unless you make alternative arrangements directly with the appropriate departments/organisation. No monies will be deducted whilst you are on unpaid leave, so you must contact the relevant body to make alternative arrangements, regarding any arrears to be paid on your return to work.

If you have any relevant agreements to repay any loans or monies owed to your Trust/UHN, you are obliged to continue these arrangements during your adoption, unless you agree alternative arrangements.

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Your adoption pay will be paid directly into your nominated bank account on normal Trust pay day. There is no requirement for you to complete your timesheet via e-Rostering whilst you are on adoption leave.

4.3.5 Sickness / Illness

If you are unable to attend work due to sickness when your adoption leave has ended, the normal contractual arrangements for sickness absence will apply.

4.3.6 Fixed Term Contracts

If you have a fixed term or rotational training contract which expires after the 11th week before the expected week of placement you will have your contract of employment extended to allow you to receive the 52 weeks adoption leave plus any pay you are entitled to, subject to meeting the other qualifying conditions. You will not be required to repay any occupational adoption pay if you are not returning to your Trust following your adoption leave due your contract having ended, as it would have done, should the adoption have not occurred.

4.3.7 Rotational Training Contracts

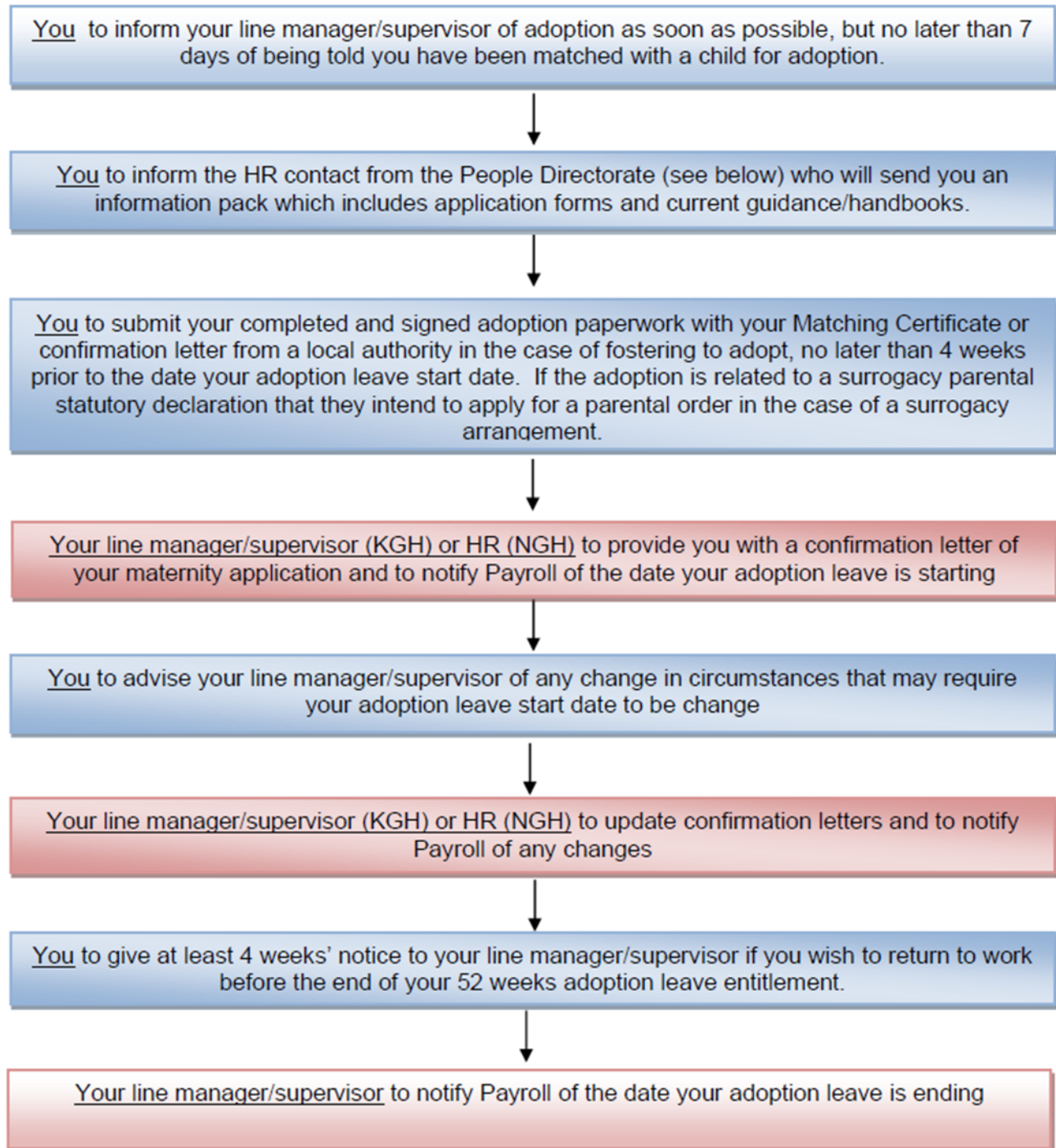
Where you are on a planned rotational training contract with appointments with one or more NHS employers as part of an agreed programme of training, you shall have the right to return to work in the same post or in the next planned post irrespective of whether your contract would otherwise have ended if the adoption had not occurred. In these circumstances your contract will be extended to enable you to complete the agreed programme of training.

In addition if you have changed your employing Trust as a requirement of your training programme and therefore do not have sufficient statutory service to be eligible for statutory adoption pay because of your rotational training programme, you will receive the value of statutory adoption pay from your Trust.

4.3.8 Process for Applying for Adoption Leave / Key Dates

The flowchart below summarises the key responsibilities of you and your line manager/supervisor/employing Trust and the key dates as part of the adoption process.

POLICY



4.3.9 More Information

You will find additional information [here](#) in the Leave to Start a Family SWAY, which covers:

- Annual leave
- Keeping in Touch Days
- Returning to Work
- Tax-free Childcare
- Day Nurseries

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You can also access further information via the intranet:

- NGH – [Maternity, Adoption, Paternity and Shared Parental Leave](#)
- KGH – [Maternity, Paternity and Adoption Leave](#)

Further assistance can be sought from:

NGH Colleagues	KGH Colleagues
People Services – Governance & Compliance Lead ngn-tr.hradvisors@nhs.net	People HR Advisors kgh-tr.humanresourcesupport@nhs.net

4.4. Shared Parental Leave

4.4.1 Shared Parental Leave Rights

Shared parental leave is a statutory entitlement which, if you are eligible, provides you with a choice as to how to share the care of your child during the first year of their life or adoption.

As a parent of a child born (including through surrogacy) or matched for adoption you may be entitled to shared parental leave.

This entitlement enables a parent who is on maternity or adoption leave to reduce/curtail their maternity/adoption leave and convert the remaining leave into shared parental leave.

This leave can then be given to your partner to opt into the shared parental leave system and take any remaining weeks/pay, if you both meet the qualifying criteria. Alternatively, it can be shared between you, and you can both decide to be off work at the same time or take it in turns with to have periods of leave to look after your child.

Unlike maternity, adoption and paternity leave, you can stop and start your shared parental leave and return to work between periods of leave as you are able to submit up to three notifications to book periods of leave.

Shared parental leave can only be used by two people, the mother/carrying parent/adopter and one of the following:

- Biological parent of the child, regardless of gender
- Spouse or partner of the person with parental responsibility (including same-sex relationships)

Both of you must share the main responsibility for the care of the child at the time of the birth or placement for adoption. You would not be eligible if one of you started sharing responsibility for the child after it was born or the placement had started in the case of adoption.

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If you qualify for shared parental leave, you have the right to:

- Shared parental leave
- Shared parental pay, if you meet the qualifying criteria
- Protection against unfair or unfavourable treatment, discrimination or dismissal
- Shared parental leave in touch days
- Protection of terms and conditions of service
- Redundancy protection from the date that your shared parental leave begins. If you take less than 6 weeks leave, the protection ends on the last day of the leave. If you take 6 or more continuous weeks, the period will end 18 months from your child's birth. If you take discontinuous leave, the protected period ends at the end of each period of shared parental leave
- The right to return to your job.
- Access to leave under the Neonatal Care (Leave and Pay) Act should an adopted baby receive neonatal care within the first 28 days of birth for at least 7 days (see section 4.1.9 – Maternity or section 4.2.7 – Paternity)

In addition, you and new applicants (internal or external) for posts within your Trust/UHN must be given equal opportunities in employment, promotion, training and development, regardless of if you are taking shared parental leave.

Your shared parental leave counts as continuous service and you retain all of your contractual rights, except your normal salary whilst you are on shared parental leave.

4.4.2 Shared Parental Leave

As an NHS employee you have the right to take up to 50 weeks of shared parental leave (minus any maternity or adoption leave taken), regardless of length of service or if you are in a contracted post or a Bank Worker.

Both of you must qualify in order to be able to access shared parental leave.

A maximum of 50 weeks shared parental leave will be available due to the compulsory maternity leave period of two weeks that must be taken immediately after the baby is born. Likewise with adoption leave there is two week period that must also be taken before any shared parental leave can commence.

Paternity leave also needs to be taken before any shared parental leave commences.

When a mother/ carrying parent/adopter gives notice to reduce/curtail their maternity/adoption entitlement the partner can take their shared leave while the mother/ carrying parent/adopter is still using their maternity/adoption leave entitlement.

Shared parental leave can start on any day of the week and is to be taken in complete weeks.

Shared parental leave must be completed by your child's first birthday or by one year anniversary of your child's placement and any leave not taken by this time will be lost.

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You can submit a maximum three separate notifications to book periods of leave, giving at least 8 weeks' notice each time, either for:

- Continuous Leave

A request for a number of consecutive weeks in a single unbroken period/block of time for example, a request for six weeks in a row, so long as it does not exceed the total number of weeks available.

You have a right to take continuous leave, and we will not refuse these requests.

- Discontinuous Leave

A request for two or more periods of a set number of weeks of leave over a period of time with, taken in separate blocks of time with breaks between the leave where you return to work in between each period, for example a request for six weeks leave spread over three months and you will work every other week.

The Trust will carefully consider requests for discontinuous leave, weighing up the potential benefits to you and against any adverse impact on service/organisation needs, however we do have the right to refuse these requests. If a request for discontinuous leave is refused you can either withdraw it within 15 days of submitting the original notification and if it is not withdrawn it will automatically default to continuous instead. However, every effort should be made to come to an alternative arrangement.

If not provided on the Shared Parental Leave/Pay Application Form, the Trust may request, within 14 days further evidence of eligibility, which can include:

- The name and business address of your partner's employer or your partner's contact details if they are self-employed or not currently employed)
- A copy of a child's birth certificate
- Evidence of an adoption.

This information must be produced with 14 days of the request in order to be entitled to shared parental leave.

If after submitting your paperwork you want to change the start date of your shared parental leave you need to give 4 weeks' notice, or as much as you can, if that is not possible, using one of your 3 available notifications to make the change (not applicable if the change is required due to your baby being born early or late, or a change to the placement date in the case of adoption).

If you sadly lose your baby, please see section 4.6 for more information on the leave options that are available.

NHS Employers have put together a guide of some examples as to how leave can be shared and you can find these [here](#).

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4.4.3 Shared Parental Pay

A maximum of 37 weeks shared parental pay may be available if the mother/ carrying parent/adopter curtails their maternity or adoption leave before they have exhausted the 9 months pay, they may be entitled to.

You may be entitled to shared parental pay from your Trust, if you meet the qualifying criteria which is based on specific eligibility criteria as detailed in the table below:

Pay Type	Qualifying Criteria	Pay Received
Occupational Shared Parental Pay (OSPP)	- Have at least 12 months continuous service (in a substantive post – excludes bank work) with one or more NHS employers at the beginning of the 11th week before the expected week of childbirth or at the beginning of the week in which they are notified of being matched with a child for adoption, or by the 15th week before the baby is due if applying via a surrogacy arrangement - Intend to return to work in the NHS (in a substantive post – excludes bank work), for at least 3 months after your shared parental leave has ended.	Paid at the same rate as occupational maternity/adoption pay (minus any pay already received through either maternity or adoption leave/pay)
Statutory Shared Parental Pay (ShPP)	- The mother/ carrying parent/adopter must have been entitled to statutory maternity or adoption pay - Confirm that the mother/carrying parent/adopter has either returned to work or given binding notice that they are bringing their maternity/adoption leave to an early end. - Must intend to care for your child during the week in which leave is payable - Must worked for your employing Trust continuously for at least 26 weeks by the end of the 15th week before the due date - Must remain in your employing Trusts employment until you start your leave. - Have average weekly earnings in the 8 weeks up to and including the qualifying week of at least equal to the Lower	Paid at the same rate as statutory maternity/adoption pay (minus pay any already received through either maternity or adoption leave/pay)

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Pay Type	Qualifying Criteria	Pay Received
	Earnings Limit for National Insurance Contributions. - Confirm the partner meets the statutory employment and earnings test of: - Have been working for at least 26 weeks out of the 66 weeks before the week the baby's due (the 26 weeks do not need to be in a row) - Have earned at least £390 in total across any 13 of the 66 weeks (add up the highest paying weeks - they do not need to be in a row)	

Your shared parental pay will be calculated on the average weekly earnings (contractual earnings which count for NI contributions) that occurs over the 'relevant period' of at least eight weeks up to and including the last pay day before the end of the qualifying week (15th week before the expected week of childbirth or the end of the week that matching took place). This period will vary depending on the frequency of when you are paid and can also be affected by additions/deductions to your contractual earnings which count for NI contributions salary during the 'relevant period'. This means if you are in a salary sacrifice scheme or have deductions for childcare vouchers your average pay calculation will be affected.

If you are entitled to occupational shared parental pay, it may be possible for this to be paid as an equal fixed amount across your shared parental leave period. However please note that choosing this option:

- Only applies to occupational shared parental pay only and not any other pay you may be entitled to, such as statutory shared parental pay. Therefore, please note that equalling out occupational shared parental pay is likely to result in your salary being different each month.
- It is not possible to equal out pension contribution deductions, so this again may result in your salary being different each month.

If you are absent due to sickness during the 'relevant period' for the average pay calculation and as a result are in a no pay or half pay situation, your average weekly earnings calculation will be based on notional full sick pay.

If you are taking two periods of shared parental leave close together and during the 'relevant period' you are in receipt of shared parental pay (such as half pay/statutory pay) or in the no pay period. Your average weekly earnings calculation will be based on notional full pay.

If you receive a pay award or increment, after the average weekly pay calculation has taken place your shared parental pay will be recalculated. You should refer to the Staff Appraisal

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Policy for full details about increments that are due to take place during the shared parental leave period. However, you should move to the increment by default, and an appraisal should take place as soon as possible after your return, if one could not be undertaken before you started your shared parental leave.

Your shared parental pay is subject to all statutory deductions (tax and national insurance) and pension, if applicable. During any unpaid shared parental leave your contributions will be accrued until you return to work. This means that upon return to work, your accrued contributions are then re-paid over the same time period as your unpaid leave.

Any other deductions from your salary, such as parking, the staff lottery or union subscriptions will also continue to be made, unless you make alternative arrangements directly with the appropriate departments/organisation. No monies will be deducted whilst you are on unpaid leave, so you must contact the relevant body to make alternative arrangements, regarding any arrears to be paid on your return to work.

If you have any relevant agreements to repay any loans or monies owed to your Trust/UHN, you are obliged to continue these arrangements during your shared parental leave, unless you agree alternative arrangements.

Your shared parental pay will be paid directly into your nominated bank account on normal Trust pay day. There is no requirement for you to complete Employee Online/e-Rostering whilst you are on shared parental leave.

4.4.4 Sickness / Illness

If you are unable to attend work due to sickness when your shared parental leave has ended, the normal contractual arrangements for sickness absence will apply.

4.4.5 Fixed Term Contracts

If you have a fixed term or rotational training contract which expires after the 11th week before the expected week of childbirth (15th week for surrogacy) or the date of matching in the case of adoption, you will have your contract of employment extended to allow you to receive the 52 weeks shared parental leave plus any pay you are entitled to, subject to meeting the other qualifying conditions. You will not be required to repay any occupational shared parental pay, if you are not returning to your Trust following your shared parental leave due your contract having ended, as it would have done.

4.4.6 Rotational Training Contracts

Where you are on a planned rotational training contract with appointments with one or more NHS employers as part of an agreed programme of training, you shall have the right to return to work in the same post or in the next planned post irrespective of whether your contract would otherwise have ended if the leave had not occurred. In these circumstances your contract will be extended to enable you to complete the agreed programme of training.

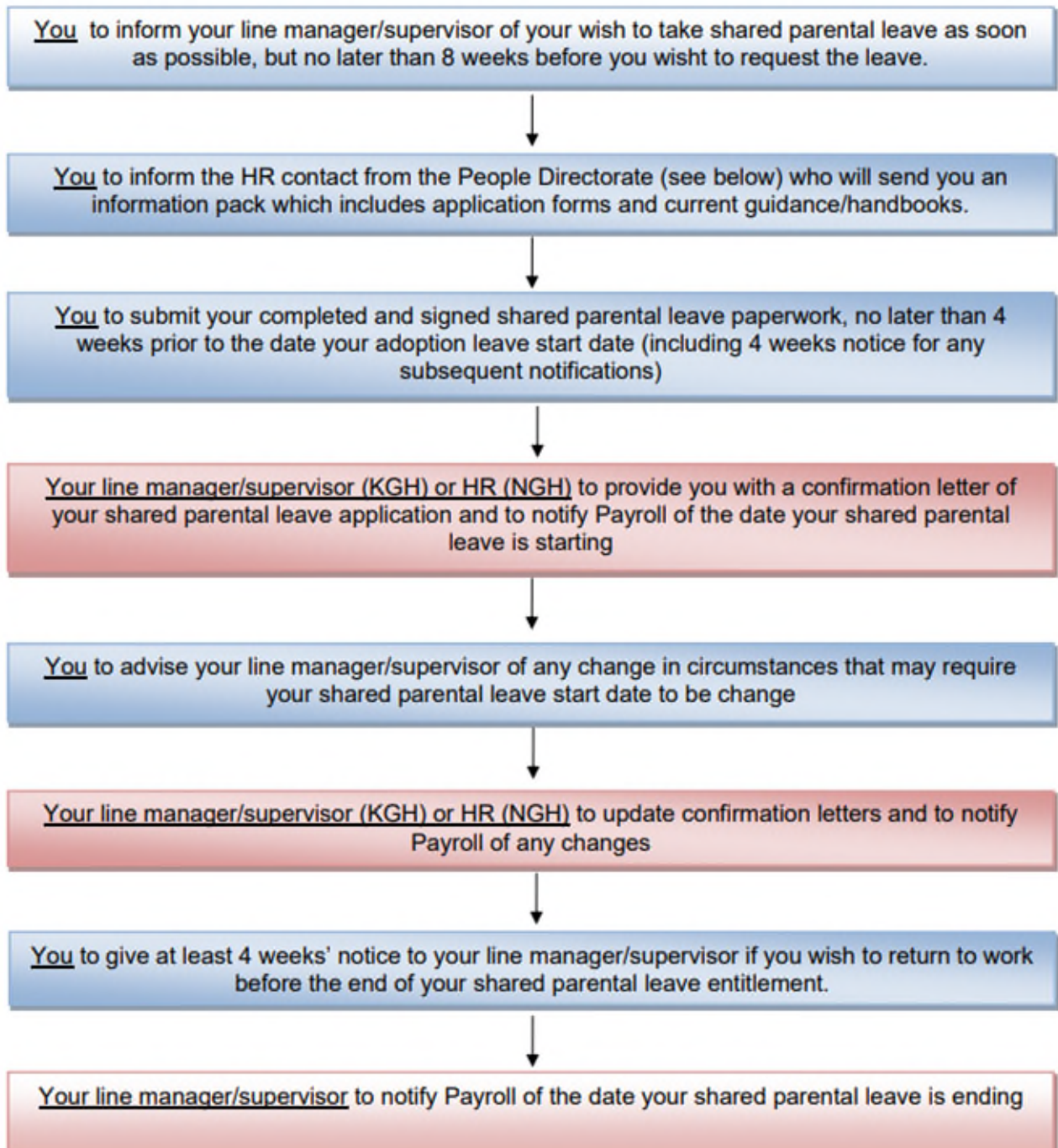
In addition if you have changed your employing Trust as a requirement of your training programme and therefore do not have sufficient statutory service to be eligible for statutory

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shared parental pay, because of your rotational training programme, you will receive the value of statutory shared parental pay from your Trust.

4.4.7 Process for Applying for Shared Parental Leave / Key Dates

The flowchart below summarises the key responsibilities of you and your line manager/supervisor/employing Trust and the key dates as part of the shared parental leave process.



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4.4.8 More Information

You will find additional information [here](#) in the Leave to Start a Family SWAY, which covers:

- Annual leave
- Keeping in Touch Days
- Returning to Work
- Tax-free Childcare
- Day Nurseries

You can also access further information via the intranet:

- NGH – [Maternity, Adoption, Paternity and Shared Parental Leave](#)
- KGH – [Maternity, Paternity and Adoption Leave](#)

Further assistance can be sought from:

NGH Colleagues	KGH Colleagues
People Services – Governance & Compliance Lead ngh-tr.hradvisors@nhs.net	People HR Advisors kgh-tr.humanresourcesupport@nhs.net

4.5. Surrogacy

If you are acting as a surrogate and carrying a child for another person or couple you will be able to access all the maternity leave entitlements, as detailed in section 4.1.

If you are having a child via a surrogacy arrangement you will be entitled to access all the entitlements associated with either:

- Adoption leave - see section 4.3
- Paternity leave - see section 4.2

In addition, once you have told us about the surrogacy you will be entitled to reasonable paid time off work to attend antenatal appointments with the surrogate, without the need to make the time up or take it as annual leave. We cannot refuse to give reasonable time off, however your line manager/supervisor is entitled to ask for evidence of the appointments from the second appointment onwards.

4.6. Baby Loss

Should you sadly suffer a loss during your pregnancy or during your maternity leave you may still be entitled to some leave and the type and length of the leave available will be dependent on when the loss occurs, more details are provided below.

Please remember that we can also help with additional health and wellbeing and/or psychological support and details of how to access these can be found [here](#).

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You can also find more information on baby loss or miscarriage from:

- [Tommy's](#)
- [Sands](#)
- Northamptonshire Sands – Helpline – 07907 836050
- [Saying Goodbye](#)

4.6.1 Miscarriage, Abortion/Termination, Ectopic Pregnancy or Molar Pregnancy

Should you experience baby loss for one of the above reasons (usually occurring before 24 weeks of pregnancy), 2 weeks paid leave will be granted to you, for each occasion. If you are the partner of someone who has experience baby loss for one of the above reasons, you will be granted up to 1 weeks paid leave, for each occasion.

Paid time off for appointments linked to pregnancy loss, for example, medical examinations, scans and tests and mental health-related interventions will also be granted beyond the initial leave detailed above.

This leave should be recorded on e-Rostering as paid authorised leave.

4.6.2 Still Birth or Death of Child

If your baby is still born (after the start of the 24th week of pregnancy) or if your baby is born alive and passes away after being born (regardless of when in the pregnancy this occurs and even if they only survive for an instant) you will be entitled to all of your maternity or paternity leave and pay entitlement. In order to process this for you we will still require the completed maternity/paternity paperwork and a copy of the MATB1 with Part B completed or a copy of the Baby Loss Certificate.

In these circumstances you may also be entitled to Child Bereavement Leave as well and you should refer to the Special Leave Policy. If you were having a child via a surrogacy arrangement you will also be entitled to Child Bereavement Leave in these circumstances.

This leave should be recorded on e-Rostering as paid authorised leave.

4.7. **IVF / Fertility Treatment**

In order to support colleagues who are undergoing IVF/fertility treatment we can offer you up to 3 days paid leave for each cycle of IVF treatment, up to a maximum of 3 cycles per year (up to 9 days maximum each year), to undergo fertility treatment. This applies equally if:

- You are undergoing implantation/fertility treatment
- You are the partner of someone undergoing implantation/fertility treatment
- You are a donor
- You are part of a same sex couple and is undergoing implantation treatment with a third party donor.

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Within this leave entitlement there is some degree of flexibility as to how and when the leave is taken, as long as you do not exceed the 9 days total per year and this is agreed with your line manager/supervisor.

If you require further time off above the beyond the 3 cycles of 3 days per year, you should discuss with your line manager/supervisor alternative options such as annual leave, unpaid leave, using time off in lieu or working the time back.

Should the treatment leave you feeling unwell or unfit for work you should report in sick to you line manager, as per the Health and Wellbeing at Work Policy (including Supporting Attendance at Work).

5. ROLES & RESPONSIBILITIES

ROLE	RESPONSIBILITY
Chief Executive and the Trust Board	Are responsible for ensuring there is a policy in place.
The Chief People Officer and Deputy Chief People Officers	Will oversee implementation of the policy and monitor compliance and effectiveness.
Managers/Supervisors	Are responsible for following the principles and processes in this policy, while ensuring: • Ensuring their own familiarity with this policy and adhering to the sections of this policy that are relevant to the type of leave being requested. • Applying this policy in a fair, consistent and compassionate manner. • Conducting a risk assessment for new and expectant colleagues or colleagues who are returning to work within 6 months of giving birth or continuing to breast/chestfeeding.
People Partnering Team	Are responsible for: • Providing advice and support to you and your line manager/supervisor in relation to this policy • Providing training • Ensuring their own familiarity with this policy and adhering to the sections of this policy that are relevant to the type of leave being requested. • Applying this policy in a fair, consistent and compassionate manner • Updating and reviewing this policy in line with statutory legislation and NHS Terms and Conditions of Service

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ROLE	RESPONSIBILITY
All Employees	Have a personal responsibility to follow the procedures detailed in this policy and to: • Informing your line manager/supervisor and the People Team of your leave request as soon as possible and in line with the requirements of this policy • For reporting any changes that have or may affect your work or ability to perform your duties • Ensuring you are familiar with and adhering to the sections of this policy that are relevant to the type of leave you are requesting. • Support UHN to achieve its Vision • Act at all times in accordance with UHN values • Follow duties and expectations of staff as detailed in the NHS Constitution – Staff Responsibilities

6. DEFINITIONS

EDC - Expected Date of Confinement	The date on which the GP/Midwife has estimated the baby will be born, as detailed on the MATB1 Form.
EWC - Expected Week of Confinement	The week in which the baby is due as detailed on the MATB1 Form. The week always starts from the Sunday prior to the expected date of confinement.
Foster to Adopt	The placing of a child with approved prospective adopters firstly as a foster child.
IVF	In Vitro Fertilisation.
MA - Maternity Allowance	May be payable to those who are not entitled to SMP, subject to the necessary qualification criteria being met. This is not paid by the employer and is claimed by the employee via the Job Centre Plus.
MATB1 Form	A certificate, which a GP/Midwife/Obstetrician will issue to an employee confirming the date their baby is due (EDC). It should not be signed more than 20 weeks before the EWC as it will be invalid.
Matching Certificate	Statement of confirmation that a person/s have been officially matched to a child for adoption.
Miscarriage	A miscarriage is the spontaneous loss of a pregnancy before 24 weeks of gestation
NI	National Insurance
OAP – Occupational Adoption Pay	Enhanced level of pay under NHS terms and conditions, to those who have the required length of NHS service.
OMP – Occupational Maternity Pay	Enhanced level of pay under NHS terms and conditions, to those who have the required length of NHS service.

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OPP – Occupational Paternity Pay	Enhanced level of pay under NHS terms and conditions, to those who have the required length of NHS service.
OSPP - Occupational Shared Parental Pay	Enhanced level of pay under NHS terms and conditions, to those who have the required length of NHS service.
SAP - Statutory Adoption Pay	The statutory amount of pay to which employees are legally entitled to, subject to meeting the qualification criteria. Colleagues are entitled to this regardless of whether they intend to return to work or not.
ShPP - Statutory Shared Parental Pay	Remaining unused Statutory Maternity/Adoption Pay that can be shared, if eligible, during shared parental leave.
SMP - Statutory Maternity Pay	The statutory amount of pay to which employees are legally entitled to, subject to meeting the qualification criteria. Colleagues are entitled to this regardless of whether they intend to return to work or not.
SMP1 Form	Produced by the Payroll Department it confirms an employee does not qualify for SMP and enables them to apply for Maternity Allowance.
SPP - Statutory Paternity Pay	The statutory amount of pay to which employees are legally entitled to, subject to meeting the qualification criteria. Colleagues are entitled to this regardless of whether they intend to return to work or not.
Still birth	A stillbirth is when a baby is born deceased after the start of 24 completed weeks of pregnancy.
Surrogacy/Surrogate	Where an individual (the surrogate) agrees, prior to conception, to carry a child for another person or couple and then hands the baby to them as agreed following the birth

7. COMPLIANCE STATEMENTS

Equality & Diversity

This document has been designed to promote equality, diversity, inclusion and human rights in line with the Trust's Equality, Diversity and Inclusion Strategies. It has also been analysed to ensure that as part of the Public Sector Equality Duty the Trust has demonstrated that it has given 'due regard' to its equality duty and that, as far as is practicable, this document is free from having a potential discriminatory or adverse/negative impact on people or groups of people who have relevant protected characteristics, as defined in the Equality Act of 2010.

The completed 'equality analysis' assessment tool must be submitted with the policy. The assessment tool must be completed by someone trained to carry out impact assessments.

NHS Constitution

The contents of this document incorporates the NHS Constitution and sets out the rights, to which, where applicable, patients, public and staff are entitled, and pledges which the NHS is committed to achieve, together with the responsibilities which, where applicable, public, patients and staff owe to one another. The foundation of this document is based on the

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Principles and Values of the NHS along with the Vision and Values of University Hospitals of Northamptonshire NHS Group.

Person Identifiable/Confidential Data and Privacy Rights

In line with the UK General Data Protection Regulation (2016) and the Data Protection Act (2018) the Trust is obliged to treat all information in a secure, professional and ethical manner, whilst keeping all person identifiable and personal data confidential. In addition, the Trust will not share employee information with a third party, unless there is a legal basis for disclosure, for example for the detection and prevention of crime, or if it is in the legitimate interest of the Trust.

As part of the Data Security and Protection policies of the Trust and data protection legislation, if the Trust is required to share any reports / information / data relating to the processes and procedures of any of our policies, the data, where possible, will be anonymised to remove person identifiable / confidential data unless there is a justifiable reason not to. It is important that policy leads are aware that policies may be released in response to FOI requests.

For further information regarding a Data Protection Impact Assessment and Sharing Personal Data, please contact the Data Security and Protection Team @ ngh-tr.data.protectionact@nhs.net OR datix.enquiries@nhs.net

8. IMPLEMENTATION & TRAINING

This policy is applicable with immediate effect, from the date that it is published on the intranet. The Governance Department will ensure that any previous versions are removed. HR People Partners/Advisors will advise managers when using the policy.

No specific training is required. Maternity presentations are run on a monthly basis for pregnant colleagues who wish to attend, and managers are also welcome, should they wish to enhance their understanding of maternity leave and entitlements.

Colleagues and managers requiring further information on any of the leave referred to in this policy can request support from the People Directorate contacts referred to in the policy.

9. MONITORING & REVIEW

Requirement of the policy and compliance to be monitored	Frequency of audit	Responsibility for audit	Results of monitoring reported at:

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Equality and Diversity Annual Workforce Report monitoring (numbers accessing maternity, paternity, adoption and shared parental leave)	Annual	Head of OD and Inclusion	People Committee
Numbers accessing paid authorised absence as part of roster compliance audits	Quarterly	Deputy Head of People Process & Planning	ILT

10. REFERENCES & ASSOCIATED DOCUMENTATION

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University Hospitals of Northamptonshire (UHN) Annual Leave & Public Holiday Entitlements for Agenda for Change Staff Policy (2023)

University Hospitals of Northamptonshire (UHN) Appraisal Policy (2025)

University Hospitals of Northamptonshire (UHN) Health and Wellbeing at Work Policy (2024)

University Hospitals of Northamptonshire (UHN) Special Leave Policy (2023)

APPENDICES

Appendix 1 - Risk Assessment Process for Colleagues who are Pregnant, Have Recently Given Birth or are Breast/Chestfeeding

Appendix 2 - Risk Assessment Form for Colleagues who are Pregnant, Have Recently Given Birth or are Breast/Chestfeeding

Appendix 3 - Equality Impact Assessment (EqIA)

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Appendix 1 - Risk Assessment Process for Colleagues who are Pregnant, Have Recently Given Birth or are Breast/Chestfeeding

Overview

As an employer we are responsible for providing a safe working environment while effectively managing risks to the health and safety of all colleagues. This includes carrying out specific individual risk assessments for you if you:

- Are pregnant
- Have given birth in the last 6 months, or
- Are currently breast/chestfeeding

When are Risk Assessments Done?

Pregnant Colleagues

A risk assessment must be done as soon as possible after you have told your line manager/supervisor that you are pregnant, using the form at Appendix 2.

The risk assessment must be reviewed on a regular basis throughout your pregnancy and also as you approach week 28 of your pregnancy. It should also be reviewed/repeated if you have any concerns at all, during your pregnancy, about risks or hazards at work that could potentially put yourself or your baby at risk.

Breast/Chestfeeding

If you will be breast/chestfeeding when you return to work after your maternity leave you should make your manager/supervisor aware ahead of your return date, so that they can organise for the risk assessment take place on your first day back at work.

Working within 6 Months of Giving Birth (including KIT Days)

If you return to work or undertake a Keeping in Touch Day within 6 months of giving birth a risk assessment should take place on your first day back at work.

Who Carries Out the Risk Assessment?

The risk assessment is to be undertaken jointly with you and somebody that understands your role, your working environment, and the tasks/duties that you undertake, for most colleagues this will be your line manager or a supervisor.

Risk assessments for Bank Workers should be carried out by a supervisor in the area that you are undertaking your bank shifts. That means if you bank in more than one area, you will require a risk assessment for each area. If necessary, your Bank Co-ordinator will liaise with the area/s you undertake your bank work to assign someone to undertake your risk assessments.

Risk assessments for Medical and Dental colleagues should be carried out by a Clinical Supervisor. If you do not have a Clinical Supervisor your area's Clinical Director should assign someone to undertake your risk assessment/s and to be a point of contact for you, for support during your pregnancy.

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What is the Purpose of the Risk Assessment?

Risk assessments are undertaken to identify if there are any hazards and assess the degree of risk that may affect the safety of you and/or your baby, whilst you are pregnant, breast/chestfeeding or having recently given birth.

The risk assessment will look at:

- Physical Hazards
- Specific Hazards
- Working Conditions
- Mental Demands of the Role

For each of these elements consideration will be given to the activities undertaken, the materials/equipment used and the environment where you work.

The specifics of what will be looked at are detailed on the Risk Assessment Form at Appendix 2.

What do we do if Risks are Identified?

If any risks are identified action must be taken to remove, reduce or control these, which can include:

- Temporarily adjusting your hours of work, with no change to normal pay
- Temporarily adjusting your duties
- Offering suitable alternative work elsewhere in the Trust (if available), with no change to your normal pay

Should no suitable control measure be identified you will be placed on paid leave from work for as long as necessary to protect the health and safety of you and your baby.

Who can Provide Further Advice if Needed?

Occupational Health and/or the Health and Safety Department can give advice if needed.

What do I do with the Risk Assessment Paperwork?

If no risks have been identified or control measures have been successfully identified for any hazards that do exist, the completed form should be placed on your personal file and a copy given to yourself.

For any risks where there are concerns the form should be sent to Occupational Health for further advice together with a Management Referral Form.

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Appendix 2 - Risk Assessment Form for Colleagues who are Pregnant, Have Recently Given Birth or are Breast/Chestfeeding

To be completed jointly by the line manager and the colleague

Colleague Name		Job Title	
Department		Due Date / Date Baby Born	
Date of Risk Assessment		Name of Manager/Supervisor	

Is the job predominantly? (please tick)

Manual / Physical Work	Clerical / Admin	Nursing / Patient Care	Medical & Dental / Patient Care	Technical / Scientific	Other (please specify)

Does the role involve...

Physical Hazards	Yes	No	Preventative or Risk Control Measures	If Yes, Detail Control Measures
Driving? For how long? How frequently?			Traveling in the course of work, and to and from the workplace, can be problematic for people who are pregnant, involving risks including fatigue, vibration, stress, static posture, discomfort and accidents. These risks can have a significant effect on the health of people who are pregnant or have recently given birth. Where travel for business is required, advice should be taken from the person's health care provider.	
Standing for long periods? For how long? How frequently?			Physiological changes during pregnancy (increased blood and systolic volume, general dilatation of blood vessels and possible compression of abdominal or pelvic veins) promote peripheral congestion while standing. Ensure seating is available and provision is made for more frequent breaks and rest. Constant sitting or constant standing is both inadvisable. It is better to alternate between the two. If this is not possible, provision should be made for breaks.	
A lot of walking? How far? How frequently?			As above	

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Manual handling (lifting, pushing, pulling etc)? How frequently? What types of weights?			There will be a greater risk of soft tissue injury due to hormonal changes and difficulties in performing tasks using best practice techniques. Take steps to reduce or avoid hazardous manual handling. Avoid long periods of handling loads or repetitive actions.	
Working at height or climbing steep steps?			Pregnant workers must not work at heights.	
The need to access areas with limited space e.g. storerooms? Which area?			Help and support based needs to be available when required to access areas with limited space.	
Will any tasks become more hazardous as the pregnancy progresses? Which tasks?			To be assessed based on the workplace environment.	
Does the role involve shift work? Which shifts? If so, does it involve working at night or into the night?			It may be necessary to adjust working hours temporarily, as well as the timing and frequency of rest breaks or change shift patterns. Expectant colleagues can still work night / on-call shifts unless there is a specific identified risk. In this instance, a medical certificate from their doctor or midwife stating that they must not work nights / on-calls, should be provided. NB - Any temporary adjustments to working hours/patterns must not result in a reduction of pay. Colleagues must receive their normal pay during this adjustment.	
Does the role involve lone working? If so, is this on site or in the community?			It may be necessary to adjust working arrangements to remove or minimise lone working. Expectant colleagues can still undertake lone working unless there is a specific identified risk.	

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Specific Hazards	Yes	No	Preventative or Risk Control Measures	If Yes, Detail Control Measures
Does any part of the job involve the use of chemicals, or potential exposure to biological agents, or potential exposure to radiation?			For work with hazardous substances, including chemicals which may cause heritable genetic damage, we are required to assess the risks to arising from such work, and where appropriate prevent or control the risks, this will include undertaking COSHH risk assessments and referring to Safety Data Sheets. Prevention of exposure must be the first priority. Where it is not appropriate to prevent the risk, control of exposure may be by a combination of technical measures, along with good work planning, housekeeping, the use of Personal Protective Equipment (PPE), adjustments to duties or redeployment.	
If so, are there any risks to them whilst they are pregnant or breast/feeding?			As above.	
Is there any exposure to vibration e.g. through the use of hand tools?			Excessive vibration should be avoided. It should be recognised that use of personal protective equipment will not protect an unborn child from a vibration hazard.	
Is there any risk to exposure to viruses, infectious outbreaks?			For work in areas where there are contagious viruses, such as COVID-19, we are required to assess the health risks to workers arising from such work, and where appropriate prevent or control the risks. Prevention of exposure must be the first priority. Where it is not appropriate to prevent the risk, control of exposure may be by a combination of control measures, along with good work planning, housekeeping, the use of Personal Protective Equipment (PPE), adjustments to duties or redeployment. IPC can be referred to for advice.	
Do they need to wear personal protective clothing? If so, will this present a problem as the pregnancy develops?			Assess any PPE available in the workplace.	

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Working Conditions (General)	Yes	No	Preventative or Risk Control Measures	If Yes, Detail Control Measures
Are there any obstacles in corridors or offices that could cause problems, e.g. in the event of a fire evacuation?			To be assessed based on the workplace environment and obstacles removed where possible. The Fire Safety Team can be referred to for advice and if necessary a Personal Emergency Evacuation Plan (PEEP) can be written if the colleague requires additional support in order to evacuate a building.	
Is there any other form of indoor air pollution, e.g. diesel fumes?			For work with hazardous substances, which include chemicals which may cause heritable genetic damage, we are required to assess the health risks to workers arising from such work, and where appropriate prevent or control the risks.	
Do they work in any areas where the temperature is not reasonable?			Pregnant workers should not be exposed to prolonged excessive heat or cold at work.	
Do they use a workstation has a workstation / display screen equipment risk assessment been done?			Has this been carried out previously within the past few months? If yes send a copy to the Health and Safety Team to review. If no one will need to be undertaken and sent to the Health and Safety Team	
Will workspace be a problem as the pregnancy develops?			To be assessed based on the workplace environment.	
Do they need to wear a uniform? If so, will this present a problem as the pregnancy develops?			Assess any alternative options/uniforms available in the workplace.	
Mental Demands of the Role	Yes	No	Preventative or Risk Control Measures	If Yes, Detail Control Measures
Does the job involve meeting challenging deadlines?			Stress factors ('stressors') (such as shift pattern, job security, workloads, etc.) and the particular factors affecting people who are pregnant need to be taken into account. If significant stressors are identified a stress risk assessment should be carried out and appropriate adjustments made.	

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Does the role involve rapidly changing priorities and demands?			As above.	
Does the role require a high degree of concentration?			As above.	

Summary / Additional Comments / Other Factors to Consider?

Is a referral to Occupational Health required? Yes / No

Colleague Signature		Date	
Manager/Supervisor Signature		Date	
Date of Next Risk Assessment			

2 nd Trimester Review (week 13)			
Colleague Signature		Date	
Manager/Supervisor Signature		Date	

2 nd Trimester Review (week 28)			
Colleague Signature		Date	
Manager/Supervisor Signature		Date	

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Appendix 3 Equality Impact Assessment (EqIA)

Equality Impact Assessment (EQIA) – Initial Assessment			
1. Division	People	2. Department	People Partnering
3. Person(s) completing this form	Sarah Kinsella	4. Contact Information	Sarah.kinsella5@nhs.net
5. Others involved		6. Start date of this assessment	21/7/25
7. What is being assessed (please tick)	New Procedural Doc ☐ Existing Procedural Doc ☒ New Service or Function ☐ Existing Service or Function ☐	8. Implementation/ effective date	October 2025
9. Name of Policy	Leave in Relation to Starting a Family v2		
10. What are the aims / objectives of policy / service this service?	To inform colleagues and managers of rights and responsibilities, and the processes and timescales they need to follow to apply for these types of leave.		
11. Who will be impacted by policy / service (please tick)	Patients ☐ Carers ☐ Public ☐ Staff ☒ Other ☐	11a. If staff are impacted, how many individuals / Which Groups of Staff are likely to be affected?	All colleagues who are either pregnant or the partner of a pregnant individual or adopting.
12. Who has been involved in the policy / service development (please tick)	Patients ☐ Carers ☐ Public ☐ Staff ☒ Other ☐	12a. If yes, who have you involved and how have they been involved	Full Groupwide consultation
13. What further consultation method(s), if any, are you proposing?	Formal consultation through Trust established Trade Union negotiation forum	14. How are any changes / amendments to the policy / service to be communicated?	UHN bulletin
Equality Impact Assessment – Initial Assessment Outcome			
15. As part of the completing the above have you or a member of UHN conducting EQIA identified any issues relating to the specific service/policy/plan in relation to gender, race, age, religion/belief, disability, sexual orientation, gender reassignment, marriage & civil partnership or pregnancy & maternity?		No	☐ EQIA Complete - If relating to a procedural document submit as part of the ratification process - If not relating to procedural documents please send completed EQIA to - KGH - kg-h-tr.diversity@nhs.net - NGH - ngh-tr.edi@nhs.net
		Yes	☒ Requires EQIA Full Assessment to be completed – see below.

Review of any previous EQIA actions

[illegible]

Review of information, equality analysis and potential actions

Protected characteristics groups from the Equality Act 2010	What do you know? Summary of data about your service-users and/or staff	What do people tell you? Summary of service-user and/or staff feedback	What does this mean? Positive / Neutral / Negative Impact identified from data and feedback (actual and potential)	What can you do? All potential actions to: • advance equality of opportunity, • eliminate discrimination, and • foster good relations
Age	20 and under 1% 21-25 5% 26-30 11% 31-35 15% 36-40 15% 41-45 12% 46-50 11% 51-55 11% 56-60 9% 61-65 7% 66-70 2% 71 and over 1%	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can access this leave if required	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all colleagues have access to their key rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Disability	No 85% Yes 6% Unknown/Prefer Not to Answer 9%	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can access this leave if required	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all colleagues have access to their key rights and entitlements

Protected characteristics groups from the Equality Act 2010	What do you know? Summary of data about your service-users and/or staff	What do people tell you? Summary of service-user and/or staff feedback	What does this mean? Positive / Neutral / Negative Impact identified from data and feedback (actual and potential)	What can you do? All potential actions to: • advance equality of opportunity, • eliminate discrimination, and • foster good relations
				under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Gender reassignment	Not recorded	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can access this leave if required	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all colleagues have access to their key rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Pregnancy and maternity	Maternity 74% Paternity 22% Adoption 1% Shared Parental Leave 3%	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can access this leave if required	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all colleagues have access to their key

Protected characteristics groups from the Equality Act 2010	What do you know? Summary of data about your service-users and/or staff	What do people tell you? Summary of service-user and/or staff feedback	What does this mean? Positive / Neutral / Negative Impact identified from data and feedback (actual and potential)	What can you do? All potential actions to: • advance equality of opportunity, • eliminate discrimination, and • foster good relations
				rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Race/ethnicity Including migrants, refugees and asylum seekers	Asian 23% Black 12% Mixed 2% Other 3% White 58% Unknown/Prefer Not to Answer 1%	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can access this leave if required	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all colleagues have access to their key rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Religion or belief	Atheism 13% Buddhism 1% Christianity 51% Hinduism 5% Islam 6%	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can access this leave if required	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all colleagues have

Protected characteristics groups from the Equality Act 2010	What do you know? Summary of data about your service-users and/or staff	What do people tell you? Summary of service-user and/or staff feedback	What does this mean? Positive / Neutral / Negative Impact identified from data and feedback (actual and potential)	What can you do? All potential actions to: • advance equality of opportunity, • eliminate discrimination, and • foster good relations
	Jainism 0% Judaism 0% Sikhism 1% Other 7% Unknown/Prefer Not to Answer 16%			access to their key rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Sex/Gender	Female 76% Male 24%	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can access this leave if required	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all colleagues have access to their key rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Sexual orientation	Bisexual 1% Gay or Lesbian 1% Heterosexual or Straight 85% Other sexual orientation not listed 0%	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all

Protected characteristics groups from the Equality Act 2010	What do you know? Summary of data about your service-users and/or staff	What do people tell you? Summary of service-user and/or staff feedback	What does this mean? Positive / Neutral / Negative Impact identified from data and feedback (actual and potential)	What can you do? All potential actions to: • advance equality of opportunity, • eliminate discrimination, and • foster good relations
	Undecided 0% Unknown/Prefer Not to Answer 11%		access this leave if required	colleagues have access to their key rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Marriage and civil partnership	Civil Partnership 2% Divorced 5% Legally Separated 1% Married 57% Single 31% Widowed 1% Unknown/Prefer Not to Answer 3%	Specific feedback is not gathered from staff on this leave	Positive – all staff regardless of protected characteristic can access this leave if required	Continue to offer advice and support to colleagues taking the leave and managers to ensure that all colleagues have access to their key rights and entitlements under statutory legislation, NHS Terms and Conditions of Service and the Equality Act 2010
Other relevant groups (e.g. carers)				

Equality Impact Assessment – Full Assessment Outcome

Assessment of overall impacts and recommendations	Launch of updated policy will provide an opportunity to educate and inform colleagues on the entitlements available to them.
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Equality Impact Assessment – Detailed list of data/community feedback that informed your EQIA

Title (of data, research or engagement)	Date	Gaps in data	Actions to fill these gaps: who else do you need to engage with? (add these to the Action Plan below, with a timeframe)

Equality Impact Assessment – Prioritised Action Plan¹

Impact identified and group(s) affected	Action planned	Expected outcome	Measure of success	Timeframe

Equality Impact Assessment – Final Assessment Outcome

As part of the completing the above have all the issues identified been addressed as part of EQIA assessment	No	☐	Please speak to Head of Organisational Development and Inclusion
	Yes	☒	EQIA Complete - If relating to a procedural document submit as part of the ratification process - If not relating to procedural documents please send completed EQIA to - KGH - kgh-tr.diversity@nhs.net - NGH - ngh-tr.edi@nhs.net

¹ These actions must now be transferred to service or business plans and monitored to ensure they achieve the outcomes identified.